Mental Health Clinical Trials, 2010–2025

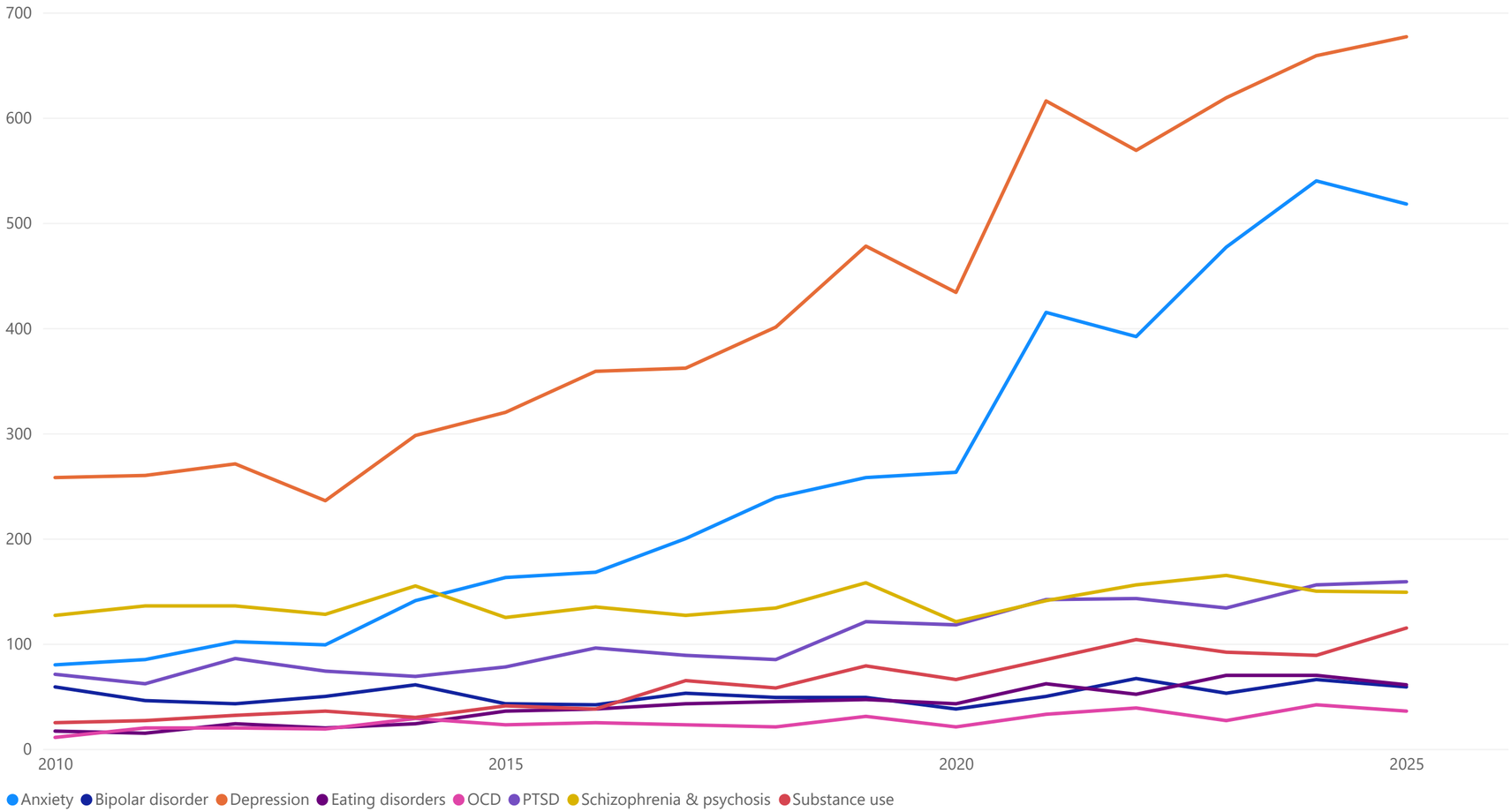
14,724 interventional trials across 8 condition groups · Source: ClinicalTrials.gov (downloaded 1 October 2026)

Condition group

All

Trials	Led by universities and hospitals	Psychedelic/MDMA trials	Include a Canadian site
14,724	83.2%	165	6.9%

Trials started per year, by condition



Key findings

- Anxiety trials increased 5.7× between 2010–12 and 2023–25, compared with only 1.2× for schizophrenia and bipolar disorder
- Eating disorders (3.6×), substance use (3.5×) and depression (2.5×) also outpaced serious mental illness
- 2020 saw fewer new trials in 7 of 8 condition groups, followed by more in all 8 the next year
- Universities and hospitals lead 83% of trials
- Use the condition filter (top right) to focus every page on one condition

Treatments

Drug, behavioural and device trials, and psychedelic research since 2010

Condition group

All

Psychedelic/MDMA trials

165

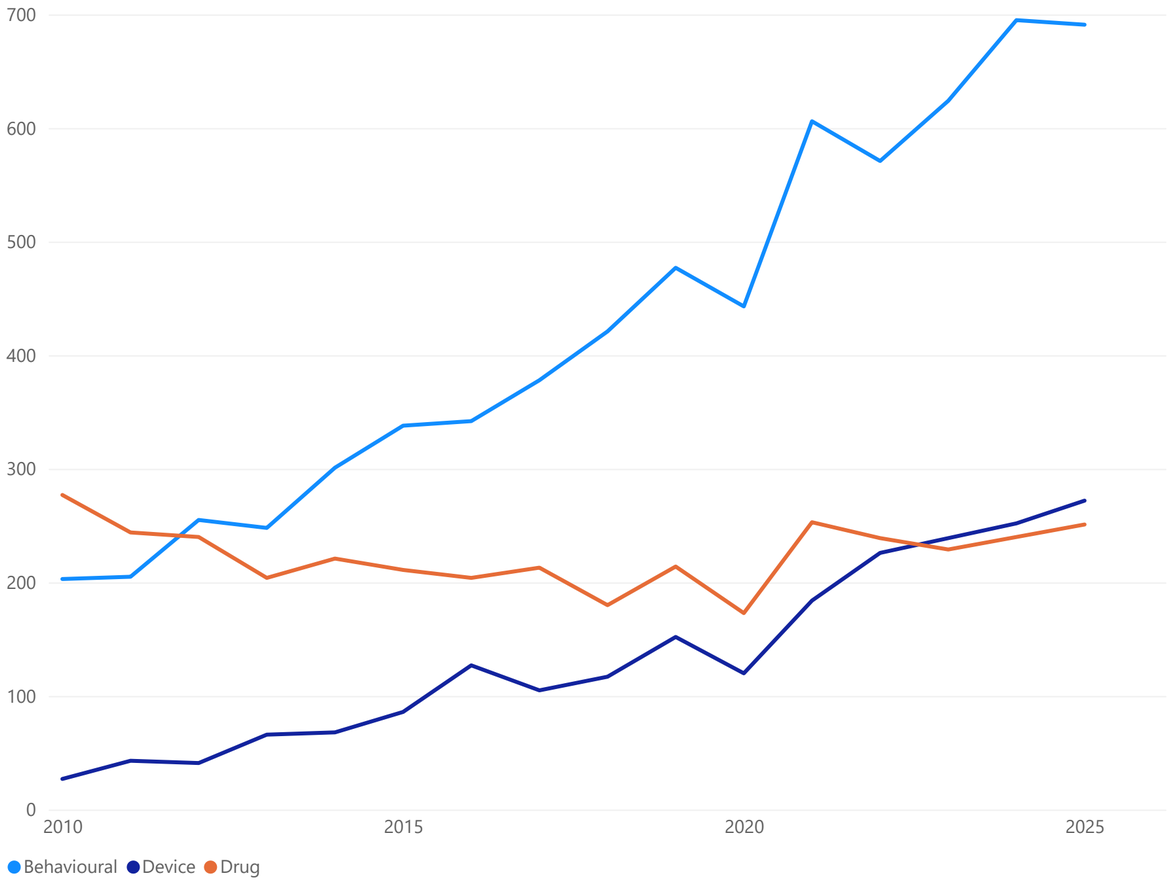
Ketamine/esketamine trials

318

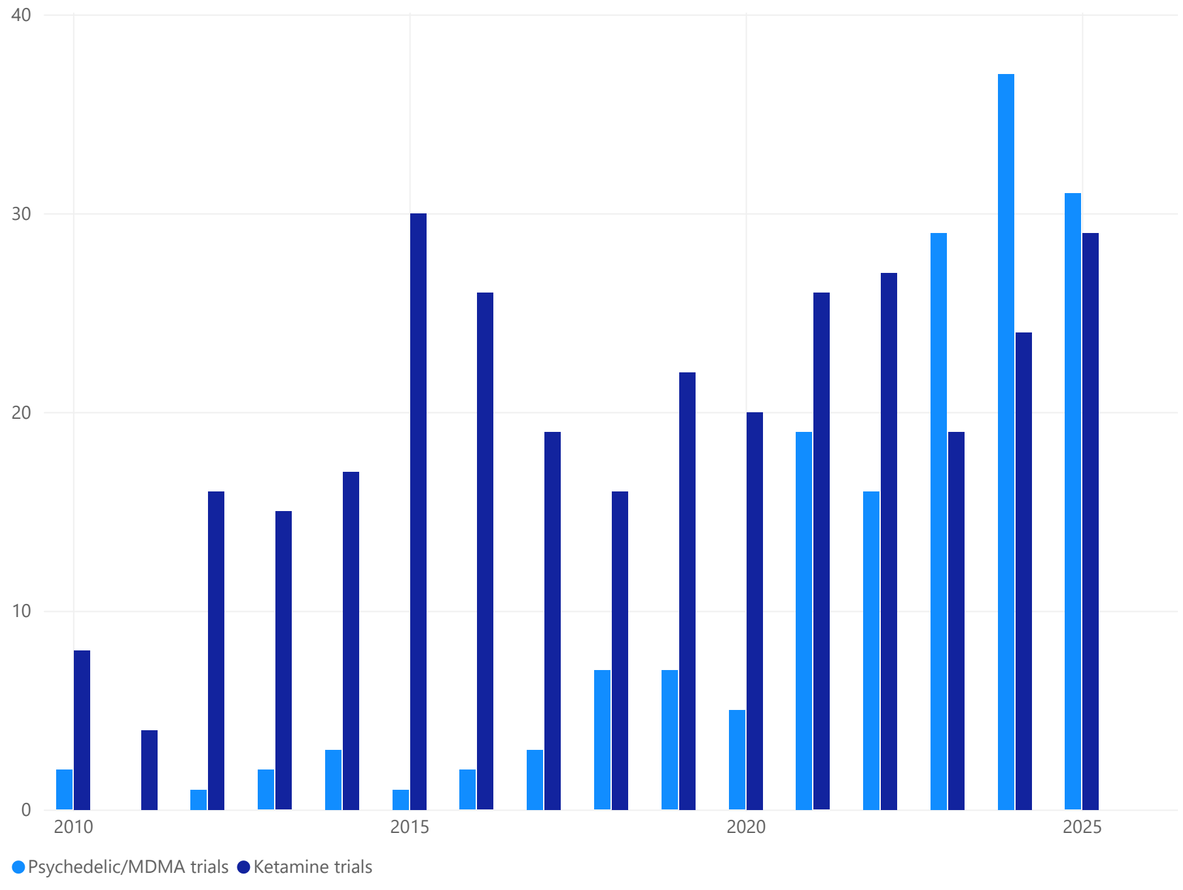
Key findings

- Drug trials held roughly level (277 in 2010, 251 in 2025) while behavioral trials increased 3.4× and device trials 10×, driven by brain stimulation and virtual reality
- 80% of psychedelic trials started in 2021 or later; they have outnumbered ketamine trials since 2023
- Placebo, control and "Other" interventions are left out of the treatment chart

Trials per year by treatment type



Psychedelic vs ketamine trials per year



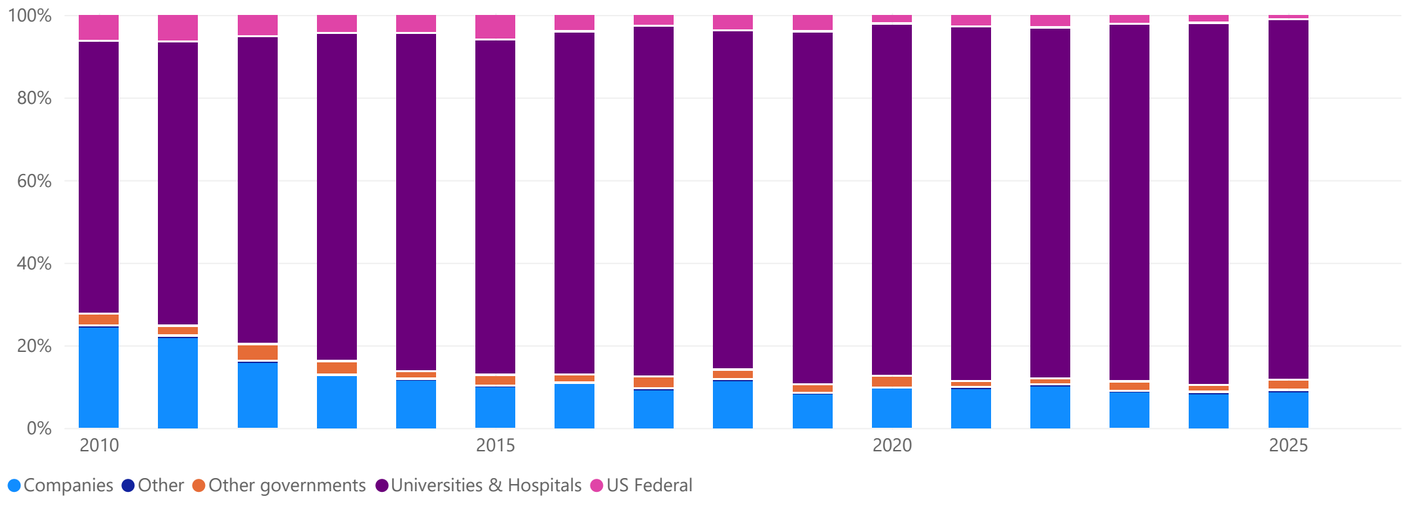
Sponsors & outcomes

Who leads mental health trials, and why some stop early

Condition group

All

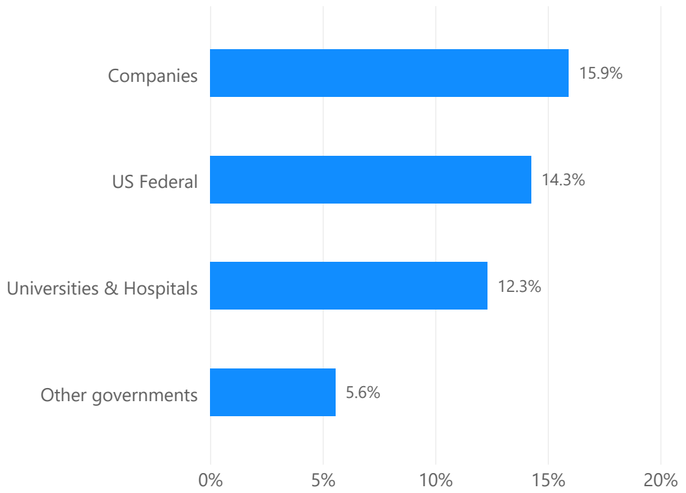
Who leads the trials? Share by sponsor type



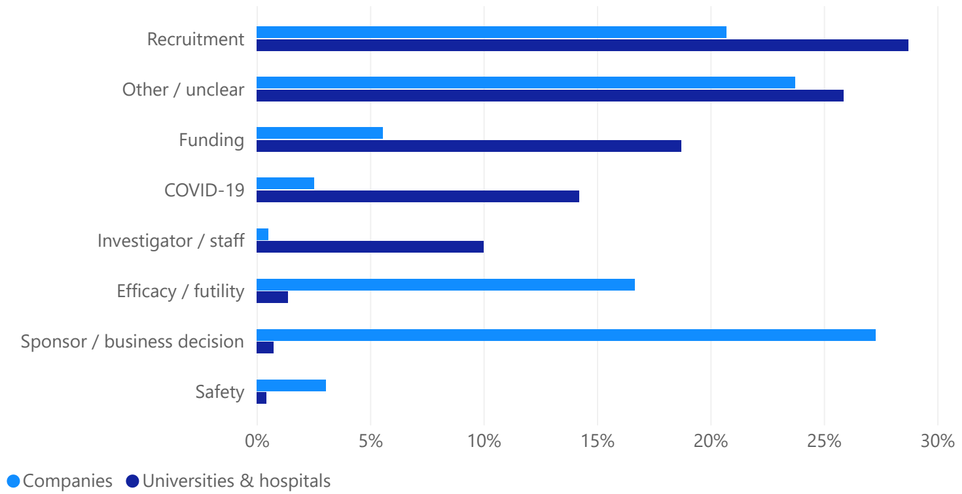
Top 10 lead sponsors

Sponsor	Type	Trials
VA Office of Research and Development	US Federal	306
Massachusetts General Hospital	Universities & Hospitals	212
Yale University	Universities & Hospitals	140
Stanford University	Universities & Hospitals	134
Centre for Addiction and Mental Health	Universities & Hospitals	133
University of California, Los Angeles	Universities & Hospitals	130
Shanghai Mental Health Center	Universities & Hospitals	116
New York State Psychiatric Institute	Universities & Hospitals	115
Karolinska Institutet	Universities & Hospitals	110
University of California, San Francisco	Universities & Hospitals	103

Share of ended trials that stopped early



Why trials stop (stated reasons, all conditions)



Key findings

- Universities and hospitals lead 87% of 2025 trials, up from 66% in 2010; company-led trials halved by 2015, then recovered to near their 2010 level
- Company trials stop early more often (15.9% vs 12.3%), mostly for business reasons or lack of efficacy; academic trials stop for recruitment, funding and COVID-19
- Safety was the stated reason in under 1% of stopped trials
- Reasons for stopping trials come from a saved SQL result, so that chart does not respond to the condition filter

Canada

Trials with at least one site in Canada

Condition group

All

Trials with a Canadian site

1,020

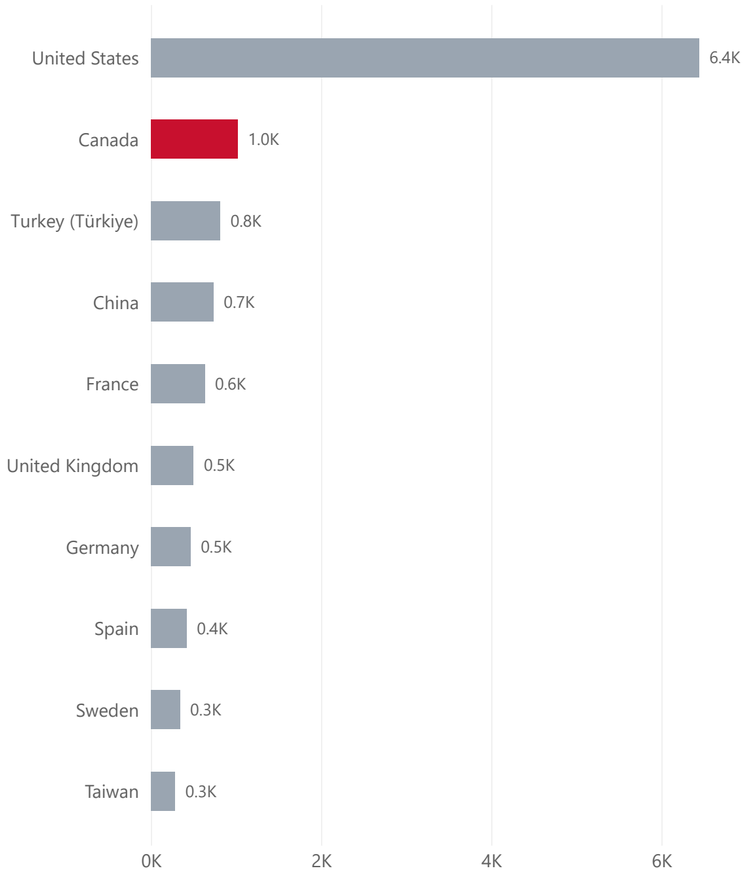
Share of all trials

6.9%

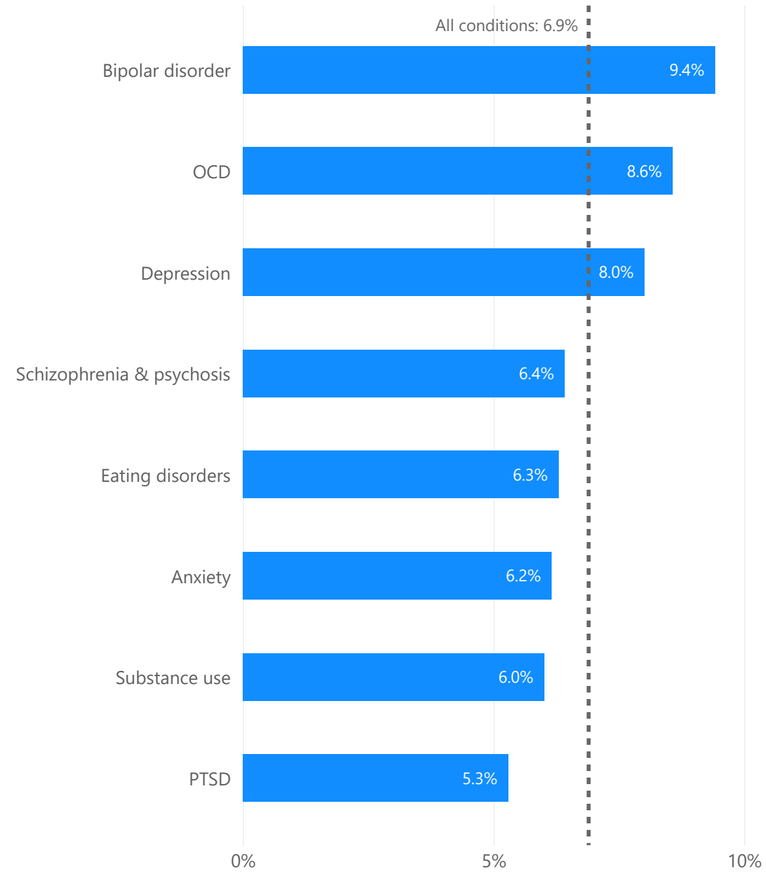
Key findings

- Canada ranks #2 after the US: 1,020 trials (6.9%) include a Canadian site, about 25 trials per million people vs 19 in the US
- Canada's share is highest in bipolar disorder, OCD and depression, and lowest in PTSD, where the U.S. Department of Veterans Affairs sponsors more trials than any other organization
- ClinicalTrials.gov is US-based, so many European and Asian trials register elsewhere

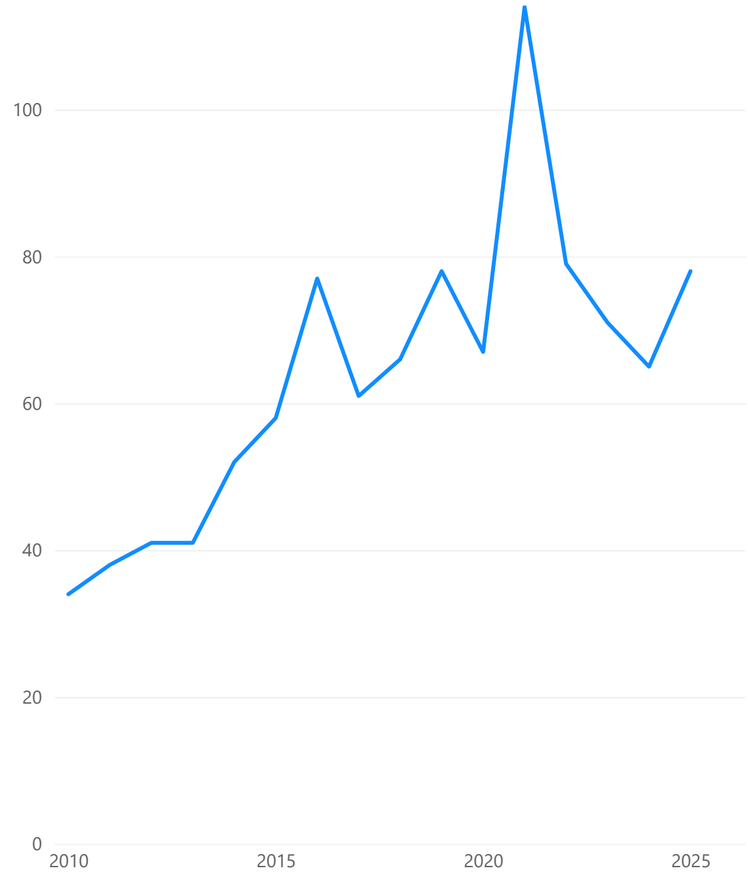
Top 10 countries by trials with a site there



Share of trials with a Canadian site, by condition



Trials with a Canadian site, per year



Methods & caveats

How the data was collected, grouped and checked

Data

Source: ClinicalTrials.gov API v2 (U.S. National Library of Medicine), downloaded 1 October 2026.

Interventional trials that started 2010–2025: 18,357 downloaded; 14,724 list at least one of the eight condition groups and are analysed here.

Pipeline: Python (API download and cleaning) → SQLite (SQL analysis) → Power BI. Code and queries: github.com/mgoldenitz/mental-health-trials-pipeline

Validation

Checked against ClinicalTrials.gov website searches: schizophrenia in 2020 matched exactly (121 vs 121); PTSD in 2022 within 4% (143 vs 149). For eating disorders in 2024, a trial-by-trial comparison showed the website's extra results were obesity and diet studies, and that no eating disorder trials were missing.

How trials are grouped

Conditions: Free-text conditions grouped into eight DSM-5-aligned groups with keyword rules; non-psychiatric uses (e.g. surgical anxiety, respiratory depression) are excluded. A trial can belong to several groups.

Treatments: ClinicalTrials.gov intervention types (Drug, Behavioural, Device); "Other" is mostly placebo and control groups and is excluded from treatment charts.

Psychedelic flag: Substance names plus company drug codes (e.g. COMP360, MM-120); ketamine/esketamine flagged separately because both are already approved.

Sponsor type: Lead sponsor only. "Companies" = industry; "US Federal" includes the VA and NIH.

Caveats

- ClinicalTrials.gov is US-based; many European and Asian trials register elsewhere, so country rankings reflect this registry.
- Registrations increased over the period, so comparisons between groups are more reliable than raw growth.
- Stop-reason chart uses grouped free text (1,213 stopped trials with a reason) and does not respond to the condition filter.
- Counts reflect registrations, not research funding or trial size.

Michael Goldenitz · Applied Data Analytics and Visualization, Sheridan College · github.com/mgoldenitz/mental-health-trials-pipeline